

India's Trusted Health Test @Home Service



NABL
ACCREDITED



Booking ID : 19852473167

Sample Collection Date : 22/Aug/2026

Amit Goyal

Male, 36 Yrs

Healthians
Relationship
Status

Preferred
Bronze
Since
2023

A Comprehensive Health Analysis Report

AI Based Personalized Report for You



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

Scan the QR using any QR code scanner

MACHINE & QC DETAILS

LABORATORY EQUIPMENT DASHBOARD

Amit Goyal

Booking ID : 19852473167



Beckman AU700

Beckman Coulter : Beckman AU700

Department - Biochemistry

No. of test(s) performed - 46

Performed Test(s): RA Test Rheumatoid Arthritis Factor, Quantitative, Amylase Enzymatic, Serum, CRP (C Reactive Protein) Quantitative, Serum, HsCRP High Sensitivity CRP, Lipase, Serum, Magnesium, Serum, Potassium, Serum, Lipid Profile Advance, Liver Function Test, Iron Studies, Kidney Function Test Advance, Apolipoproteins Profile (A1, B, B/A1 Ratio)

Description: The Beckman Coulter state-of-the-art clinical chemistry analyzer manufactured in USA, delivers exceptional performance with a throughput of up to 800 tests per hour using spectrophotometry and 1,200 tests per hour with ISE, ensuring outstanding diagnostic accuracy and precision. Utilizing advanced spectrophotometric technology, it provides reliable and highly accurate results, supporting over 21 parameters that meet Six Sigma performance standards for superior laboratory efficiency and quality.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Beckman DXI800

Beckman Coulter : Beckman DXI800

Department - Immunology

No. of test(s) performed - 10

Performed Test(s): CA-19.9 (Pancreatic Cancer Marker Test), CEA-Carcino Embryonic Antigen (Colorectal Cancer Marker Test), Ferritin, PSA-total Prostate Specific Antigen (Prostate Cancer Marker Test), Testosterone Total, Vitamin B12 Cyanocobalamin, Vitamin D Total-25 Hydroxy, Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive)

Description: The Beckman Coulter UniCel Dxl Immunoassay System manufactured in USA, utilizes chemiluminescent technology to deliver high precision and sensitivity, with a throughput of 400 tests per hour for efficient and reliable immunoassay testing.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Beckman DXH800

Beckman Coulter : Beckman DXH800

Department - Hematology

No. of test(s) performed - 5

Performed Test(s): Complete Blood Count Advance

Description: The Beckman Coulter hematology analyzer manufactured in USA, features automated intelligent morphology screen with advanced algorithms for accurate cell population analysis. It delivers highly reliable RBC, PLT, and WBC differentials through near native-state cellular characterization. Utilizing VCS 360, Data Fusion, and enhanced Coulter Principle technology, it ensures precise, reportable WBC differential results with exceptional diagnostic reliability.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Horiba Yumi2500

HORIBA : Horiba Yumi2500

Department - Hematology

No. of test(s) performed - 21

Performed Test(s): Complete Blood Count Advance

Description: The fully automated high-end hematology analyzer manufactured in France, incorporates the patented Double Hydrodynamic Sequential System (DHSS) to ensure precise, linear cell flow for superior accuracy. It provides total WBC counts using impedance and flow cytometry, and supports detailed immature cell analysis—including immature granulocytes, monocytes, lymphocytes, and large immature cells—for effective sepsis screening and cancer pre-screening. The analyzer also features an optical platelet mode for enhanced clarity in cases of microcytosis, macro platelets, or RBC/PLT interferences, along with 360° homogeneous sample mixing to minimize manual intervention and preanalytical errors.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)



Transasia Cube 80

Transasia : Transasia Cube 80

Department - Hematology

No. of test(s) performed - 1

Performed Test(s): ESR Automated

Description: The Cube 80 manufactured in Italy, is a fully automated bench-top analyzer designed for the direct determination of ESR. With its automation, flexibility, and reliability, the Cube 80 provides accurate and efficient ESR analysis suited for high-throughput laboratory environments.

Frequency of Quality Control (QC) : Twice a day (Industry best and as per NABL guidelines)

HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Amit Goyal,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

89
Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 6.82 μ IU/mL

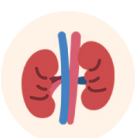
● Concern



Cholesterol Total

201.9 mg/dl

● Concern



Kidney Function

Serum Creatinine : 0.74 mg/dl

● Everything looks good



Vitamin D

29.41 ng/ml

● Concern



HbA1c

4.8 %

● Everything looks good



Vitamin B12

141 pg/ml

● Concern



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 47.7 U/L

● Everything looks good



Calcium Total

10.3 mg/dl

● Everything looks good



Iron studies

Serum Iron : 78.3 μ g/dl

● Everything looks good



Complete Hemogram

Haemoglobin (HB) : 16.9 g/dL

● Everything looks good





New Features

Report Summary

Amit Goyal

Booking ID 19852473167 | Sample Collection Date: 22/Aug/2026

Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Summary of Your Health Test Results

Hello Amit Goyal, thank you for sharing your recent health test results. It is completely normal to feel a bit concerned when seeing values outside the typical range, but please know that these results are simply indicators that provide us with a roadmap to improve your well-being. Your report suggests some areas related to your nutritional status, thyroid function, and cardiovascular markers that would benefit from some focused attention and lifestyle adjustments.

Many of these findings are interconnected and often respond very well to consistent changes in diet, hydration, and activity levels. By addressing these imbalances now, you are taking a proactive step toward feeling more energized and supporting your long-term health. We can look at these results as a helpful guide to fine-tuning your daily habits to help your body function at its best.

Suggestions for Your Health

Here are some suggestions to help you address the areas identified in your recent health tests:

- Nutritional Support and Supplementation:** Focus on incorporating more B12-rich foods such as lean meats, eggs, and fortified cereals into your diet. Given the findings regarding your vitamin levels, it is important to discuss appropriate supplementation with a healthcare provider to ensure your levels return to an optimal range, which will also help in managing your homocysteine levels.
- Thyroid and Metabolic Wellness:** Your thyroid marker suggests a need for a follow-up evaluation to understand how your metabolism is currently functioning. Prioritize consistent sleep patterns and stress-reduction techniques, as these can significantly influence thyroid health and overall hormonal balance.
- Cardiovascular and Hydration Habits:** To support your lipid profile and blood markers, focus on a heart-healthy diet rich in fiber, whole grains, and healthy fats like those found in nuts and olive oil. Additionally, ensure you are drinking adequate water throughout the day, as proper hydration is essential for maintaining healthy blood volume and circulation.
- Lifestyle and Activity Adjustments:** Incorporating regular, moderate physical activity-such as brisk walking or swimming-can help improve your cholesterol levels and support healthy blood flow. Pairing this with a diet lower in processed sugars and saturated fats will provide the necessary support for your body to naturally regulate these markers over time.

HEALTH ANALYSIS

HISTORICAL CHARTS

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

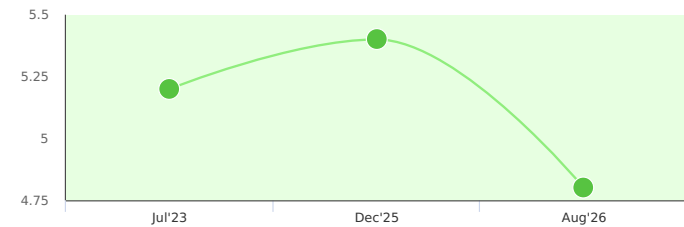
Glycated Hemoglobin (HbA1c)

Everything looks good

Your Latest result

4.8 %

22nd Aug 2026



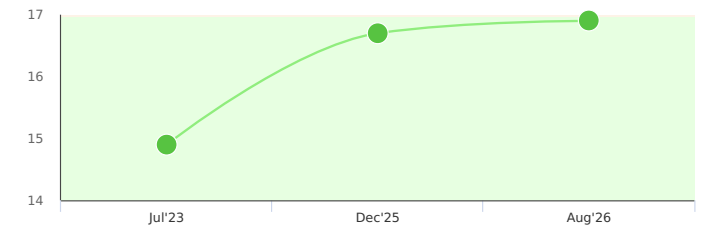
Hemoglobin Hb

Everything looks good

Your Latest result

16.9 g/dL

22nd Aug 2026



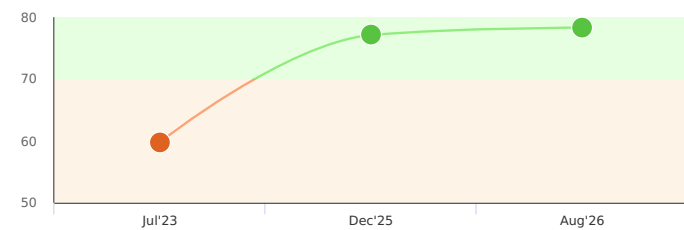
Iron, Serum

Everything looks good

Your Latest result

78.3 ug/dl

22nd Aug 2026



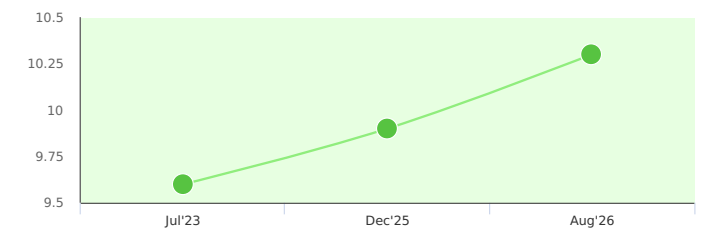
Calcium Total, Serum

Everything looks good

Your Latest result

10.3 mg/dl

22nd Aug 2026



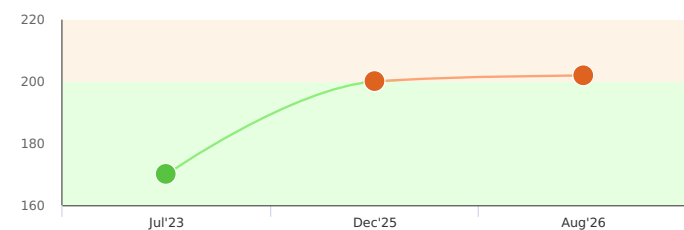
Cholesterol-Total, Serum

Concern

Your Latest result

201.9 mg/dl

22nd Aug 2026



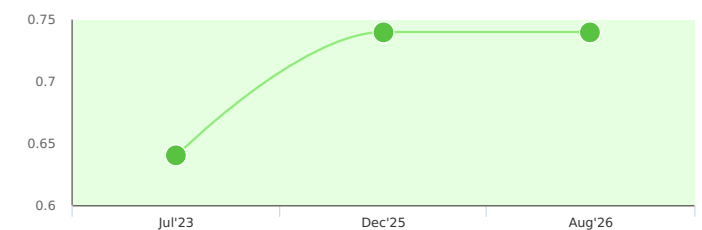
Creatinine, Serum

Everything looks good

Your Latest result

0.74 mg/dl

22nd Aug 2026



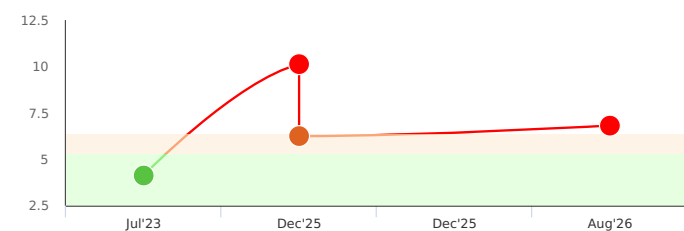
TSH Ultra - Sensitive

Concern

Your Latest result

6.82 µIU/mL

22nd Aug 2026



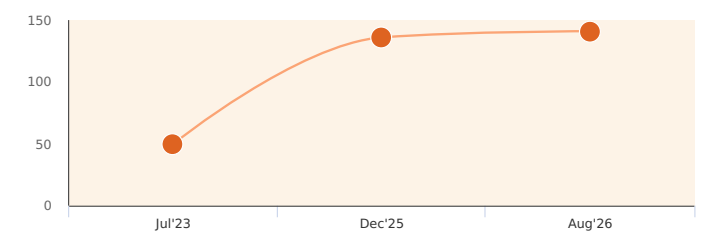
Vitamin B12 Cyanocobalamin

Concern

Your Latest result

141 pg/ml

22nd Aug 2026



HEALTH ANALYSIS
COMPARATIVE CHARTS

Amit Goyal
Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :35 (As per latest result) 29 Everything Looks Good 1 Borderline 5 Concern

Test Name	Risk Area	2 Years Ago			3 Days Ago		8 Months Ago	Latest Result 22 Aug 2026
		Booking ID 8390833383 03 Jul 2023 11:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM	Booking ID 16154005984 13 Dec 2025 12:00 PM				
Cholesterol–Total, Serum 0 –200 Normal Range		170 mg/dl	↑ 200 mg/dl	-				↑ 201.9 mg/dl High (Concern)
HDL Cholesterol Direct 40 –60 Normal Range	--	48 mg/dl	↓ 45.7 mg/dl	-				↑ 55.9 mg/dl Everything Looks Good
LDL Cholesterol –Direct 0 –100 Normal Range		107.2 mg/dl	-	-				↑ 131.9 mg/dl High (Concern)
Triglycerides, Serum 0 –150 Normal Range	--	76 mg/dl	↑ 158.5 mg/dl	-				↓ 79.7 mg/dl Everything Looks Good
VLDL 0 –30 Normal Range	--	-	-	-				14 mg/dl Everything Looks Good
Alkaline Phosphatase, Serum 43 –115 Normal Range	--	61.1 u/L	↑ 80.1 u/L	-				↑ 84.8 u/L Everything Looks Good
GGTP (Gamma GT) 5 –55 Normal Range	--	14.9 u/L	↑ 26.7 u/L	-				↑ 26.9 u/L Everything Looks Good
Proteins, Serum 6.6 –8.3 Normal Range	--	6.41 g/dl	↑ 7.05 g/dl	-				↑ 7.7 g/dl Everything Looks Good
SGOT/AST 3 –50 Normal Range	--	22.1 u/L	↑ 29.4 u/L	-				↑ 31.3 u/L Everything Looks Good
SGPT/ALT 3 –50 Normal Range	--	22.4 u/L	↑ 42.9 u/L	-				↑ 47.7 u/L Everything Looks Good
ESR Automated 0 –10 Normal Range	--	-	9 mm/1st hour	-				↓ 4 mm/1st hour Everything Looks Good
Hemoglobin Hb 13.0 –17.0 Normal Range	--	14.9 g/dl	↑ 16.7 g/dl	-				↑ 16.9 g/dl Everything Looks Good

HEALTH ANALYSIS
COMPARATIVE CHARTS

Amit Goyal
Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time ↓ Decrease From Last Time

Total Parameters :35
(As per latest result)

29 Everything Looks Good

1 Borderline

5 Concern

Test Name	Risk Area	2 Years Ago			3 Days Ago		8 Months Ago	Latest Result 22 Aug 2026
		Booking ID 8390833383 03 Jul 2023 11:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM	Booking ID 16154005984 13 Dec 2025 12:00 PM				
Platelet Count Thrombocyte count 150 -410 Normal Range	--	313 10^3/μl	↑ 324 10^3/μl	-				↑ 343 10^3/μl Everything Looks Good
T3, Total Tri Iodothyronine 0.87 -1.78 Normal Range	--	1.13 ng/ml	↑ 1.43 ng/ml	-				↑ 1.52 ng/ml Everything Looks Good
T4, Total Thyroxine 5.48 -14.28 Normal Range	--	10.42 ug/dl	↓ 9.77 ug/dl	-				↑ 12.1 ug/dl Everything Looks Good
TSH Ultra - Sensitive 0.38 -5.33 Normal Range		4.15 μIU/mL	↑ 10.12 μIU/mL	↓ 6.25 μIU/mL				↑ 6.82 μIU/mL High (Concern)
Iron, Serum 70 -180 Normal Range	--	59.8 ug/dl	↑ 77.1 ug/dl	-				↑ 78.3 ug/dl Everything Looks Good
Calcium Total, Serum 8.8 -10.6 Normal Range	--	9.6 mg/dl	↑ 9.9 mg/dl	-				↑ 10.3 mg/dl Everything Looks Good
Chlorides, Serum 101 -109 Normal Range	--	106 mmol/L	↓ 104 mmol/L	-				↓ 103 mmol/L Everything Looks Good
Creatinine, Serum 0.6 -1.6 Normal Range	--	0.64 mg/dl	↑ 0.74 mg/dl	-				0.74 mg/dl Everything Looks Good
Phosphorus-Inorganic, Serum 2.5 -4.5 Normal Range	--	3.8 mg/dl	↑ 4.2 mg/dl	-				↓ 3.9 mg/dl Everything Looks Good
Sodium, Serum 136 -146 Normal Range	--	144 mmol/L	144 mmol/L	-				144 mmol/L Everything Looks Good
Urea, Serum 17 -43 Normal Range	--	19 mg/dl	↑ 21.2 mg/dl	-				↓ 20 mg/dl Everything Looks Good
Uric Acid, Serum 3.5 -7.2 Normal Range	--	4.6 mg/dl	↑ 5.1 mg/dl	-				5.1 mg/dl Everything Looks Good

HEALTH ANALYSIS

COMPARATIVE CHARTS

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

↑ Increase From Last Time

↓ Decrease From Last Time

Total Parameters :35
(As per latest result)

29 Everything Looks Good

1 Borderline

5 Concern

Test Name	Risk Area	2 Years Ago	3 Days Ago	8 Months Ago	Latest Result 22 Aug 2026
		Booking ID 8390833383 03 Jul 2023 11:30 AM	Booking ID 16124726558 10 Dec 2025 09:00 AM	Booking ID 16154005984 13 Dec 2025 12:00 PM	
URINE KETONE Negative -Negative Normal Range	--	-	Negative	-	Negative
Glycated Hemoglobin (HbA1c) 4.2 -5.7 Normal Range	--	5.2 %	↑ 5.4 %	-	↓ 4.8 % Everything Looks Good
Apolipoproteins A1, Serum 105 -175 Normal Range	--	-	-	-	124.5 mg/dl Everything Looks Good
Apolipoproteins B, Serum 60 -140 Normal Range	--	-	-	-	93.9 mg/dl Everything Looks Good
Potassium, Serum 3.5 -5.5 Normal Range	--	-	4.08 mmol/L	-	↑ 4.27 mmol/L Everything Looks Good
Ferritin 23.9 -336.2 Normal Range	--	69.2 ng/ml	-	-	↑ 70.7 ng/ml Everything Looks Good
Vitamin D Total-25 Hydroxy 30 -100 Normal Range		9.36 ng/ml	↑ 18.23 ng/ml	-	↑ 29.41 ng/ml Low (Concern)
CRP (C Reactive Protein) Quantitative, Serum 0 -5 Normal Range	--	0.78 mg/L	-	-	↑ 2.71 mg/L Everything Looks Good
HsCRP High Sensitivity CRP 0 -1.0 Normal Range		0.72 mg/L	↑ 3.08 mg/L	-	↓ 2.3 mg/L High (Concern)
Amylase Enzymatic, Serum 22 -80 Normal Range	--	50.3 u/L	↑ 113.9 u/L	-	↓ 60 u/L Everything Looks Good
Lipase, Serum 0 -67 Normal Range	--	-	152 u/L	-	↓ 38 u/L Everything Looks Good

Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:27PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:15PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY HBA1C

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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HbA1c - Glycosylated Hemoglobin

HbA1c (Glycosylated Hemoglobin)	4.80	%	4.2 - 5.7
Method: HPLC			
Average Estimated Glucose	91.06		
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

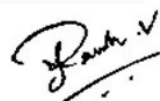
GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7
5.7 – 6.4
>= 6.5
Age > 19 Years
Goals of Therapy: < 7.0
Actions Suggested: >8.0
Age < 19 Years
Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
 - HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
 - Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapson, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
 - HbA1c may be increased in patients with polycythemia or post-splenectomy.
 - Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
 - Trends in HbA1c are a better indicator of diabetic control than a solitary test. 7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
 - HbA1c target in pregnancy is to attain level <6 % .
 - HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023



DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:06PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:23PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Sodium Fluoride	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

Glucose, Fasting	89.3	mg/dl	70 - 100
Method: Hexokinase			
Machine: BECKMAN DXC 700AU			

American Diabetes Association Reference Range :

Normal	: < 100 mg/dl
Impaired fasting glucose(Prediabetes)	: 100 - 126 mg/dl
Diabetes	: >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis

A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.


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Patient Name	: Amit Goyal	Barcode	: D0342237
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 09:13PM
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Amylase - Serum

AMYLASE	60.0	U/L	22 - 80
Method: CNPG3			
Machine: BECKMAN DXC 700AU			

Amylase is produced by exocrine pancreas and also by the salivary glands. it is used to evaluate pancreatic function and also used in the diagnosis and management of pancreatitis.

Diseases resulting in elevation of plasma alpha-amylase include: acute pancreatitis, parotitis, alcoholism, renal insufficiency and diseases such as viral hepatitis, AIDS, abdominal typhoid, sarcoidosis and trauma to the upper abdomen. There is also a detectable increase in amylase after an ERCP procedure. In acute pancreatitis, amylase increases 5-6 hours after the onset of symptoms and remains elevated for 2-5 days. The increase in plasma activity does not reflect disease severity and conversely, extensive destruction of the pancreas may not cause a significant increase in the plasma concentration of pancreatic alpha-amylase.

Lipase - Serum

LIPASE	38	U/L	<67
Method: Colorimetric			
Machine: BECKMAN DXC 700AU			

Lipase is produced in the acinar cells of the pancreas and is responsible for the hydrolysis of water-insoluble long chain fatty acid esters of glycerol. Lipase measurement in serum and plasma is used exclusively for the investigation of pancreatic disorders, usually pancreatitis. Serum lipase may be elevated in acute pancreatitis, acute episodes of chronic pancreatitis and obstructive pancreatitis

Slight elevations are also frequently present in diabetic ketoacidosis, viral hepatitis, epidemic parotitis, abdominal typhoid and sarcoidosis, due to involvement of the pancreas.

Apolipoproteins profile (A1,B,B/A1 ratio)

APO-A1	124.5	mg/dl	105 - 175
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			
APO-B	93.9	mg/dl	60 - 140
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			
Apo B /Apo A-1 Ratio	0.75		

Apolipoprotein A (apo-A; also known as Apo A-1) is the major protein (90%) of HDL. Apolipoprotein B (apo B) is major protein component of low-density lipoprotein (LDL) and is important in regulating cholesterol synthesis and metabolism.

Use:

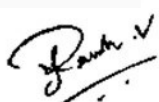
To evaluate the risk of CAD: Levels of Apo A-1 are inversely associated with premature cardiovascular disease and peripheral vascular disease. The ratio of Apo B to Apo A has greater sensitivity and specificity for CAD than individual lipid or lipoproteins. To evaluate atherosclerotic disease and to detect Tangier disease.

Apo B/A-1 ratio:

Low risk: Male: 0.4 -0.99, Female: 0.3 -0.89

Average risk: Male: 1.0-1.59, Female: 0.9-1.49

Twice average risk: Male: >1.6, Female: >1.5


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HEALTHIANS LABS



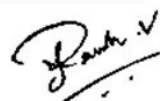
Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 09:13PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Ref. Wallach's Interpretation of Diagnostic Tests Pathways to Arriving at a Clinical Diagnosis TENTH EDITION



DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:45PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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C-Reactive Protein (CRP) -Quantitative

C-REACTIVE PROTEIN (CRP) (QUANTITATIVE)	2.71	mg/L	<5
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Method: Immunoturbidimetric
Machine: BECKMAN DXC 700AU

C-reactive protein (CRP) is one of the most sensitive acute-phase reactants for inflammation. Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. Unlike ESR, CRP levels are not influenced by hematologic conditions such as anemia, polycythemia etc.

Increased levels are consistent with an acute inflammatory process. After onset of an acute phase response, the serum CRP concentration rises rapidly (within 6-12 hours and peaks at 24-48 hours) and extensively. Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).

hs CRP (C-Reactive Protein high sensitive)

HS-CRP (HIGH SENSITIVITY C-REACTIVE PROTEIN)	2.30	mg/L	< 1.0
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Method: Latex Particle Immunoturbidimetry
Machine: BECKMAN DXC 700AU

High sensitivity C-reactive protein, when used in conjunction with other clinical laboratory evaluation of acute coronary syndromes, may be useful as an independent marker of prognosis for recurrent events in patients with stable coronary disease or acute coronary syndrome. Hs-CRP levels should not be substituted for assessment of traditional cardiovascular risk factors. Patients with persistently unexplained, marked elevation of hs-CRP after repeated testing should be evaluated for non-cardiovascular etiologies.

Clinical significance :

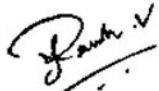
Hs-CRP measurements may be used as an independent risk marker for the identification of individuals at risk for future cardiovascular disease. Elevated CRP values may be indicative of prognosis of individuals with acute coronary syndromes, and may be useful in the management of such individuals.

Iron study

Serum Iron	78.3	ug/dl	70 - 180
Method: TPTZ Machine: BECKMAN DXC 700AU			
UIBC	294.80	ug/dl	155 - 355
Method: Nitroso-PSAP Machine: BECKMAN DXC 700AU			
Serum Total Iron Binding Capacity (TIBC)	373.1	µg/dl	250 - 400
Method: Calculated			
Transferrin Saturation %	20.99	%	10 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin,


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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
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estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	<10	IU/mL	<14
Method: Immunoturbidimetry			
Machine: BECKMAN DXC 700AU			

The rheumatoid factor (RF) test is primarily used to help diagnose rheumatoid arthritis (RA) and to help distinguish RA from other forms of arthritis or other conditions that cause similar symptoms. A cyclic citrullinated peptide (CCP) antibody test can help diagnose RA in someone who has joint inflammation with symptoms that suggest but do not yet meet the criteria of RA and may be ordered along with RF or if the RF result is negative. The RF test must be interpreted in conjunction with a person's symptoms and clinical history. A negative RF test does not rule out RA.

About 20% of people with RA will have very low levels of or no detectable RF. In these cases, a CCP antibody test may be positive and used to confirm RA. Positive RF test results may also be seen in 1-5% of healthy people and in some people with conditions such as: Sjogren syndrome, scleroderma, systemic lupus erythematosus (lupus), sarcoidosis, endocarditis, tuberculosis, syphilis, HIV/AIDS, hepatitis, infectious mononucleosis, cancers such as leukemia and multiple myeloma, parasitic infection, or disease of the liver, lung or kidney.

Potassium - Serum

Serum Potassium	4.27	mmol/L	3.5 - 5.5
Method: ISE (Indirect)			
Machine: BECKMAN DXC 700AU			

Potassium is a primary intracellular ion, only 2 % is extracellular, high intracellular concentration is maintained by a Na- K ATPase pump, which continuously transports potassium into the cell against a concentration gradient. The pump is a critical factor in maintaining and adjusting the ionic gradients, on which nerve impulse transmission and contractility of cardiac and skeletal muscle depends. In acidemia, potassium moves out of the cells, in alkalemia, potassium moves into the cells. Hypokalemia inhibits aldosterone production and hyperkalemia stimulates aldosterone production.

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Magnesium -Serum

MAGNESIUM	2.1	mg/dl	1.8 - 2.6
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Method: Xylidyl blue, tris buffer

Machine: BECKMAN DXC 700AU

Test Interpretation

Magnesium is primarily an intracellular ion associated with GI absorption and renal excretion.

It helps in the diagnosis and monitoring of hypomagnesemia and hypermagnesemia especially in renal failure or GI disorders.

Increased levels

seen in patients taking diuretics, antacids, laxatives, parenteral nutrition, magnesium for eclampsia or premature labor, renal failure, dehydration with diabetic coma before treatment, Hypothyroidism.

Decreased levels

seen in malabsorption, abnormal loss of GI fluids, chronic pyelonephritis, renal tubular acidosis, antibiotics, phosphate depletion, extra cellular fluid volume expansion. Nutritional causes -starvation, Alcoholism. Endocrine causes -hyperthyroidism, hypoparathyroidism. Metabolic causes- excessive lactation, third trimester of pregnancy. Other causes- severe burns, lytic tumors of bone, toxemia of pregnancy, sepsis.



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Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile Advance			
Total Cholesterol Method: Enzymatic Machine: BECKMAN DXC 700AU	201.9	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Serum Triglycerides Method: Enzymatic Machine: BECKMAN DXC 700AU	79.7	mg/dl	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : >= 500
Serum HDL Cholesterol Method: Enzymatic immuno inhibition Machine: BECKMAN DXC 700AU	55.9	mg/dl	40 - 60
Serum LDL Cholesterol Method: Enzymatic Machine: BECKMAN DXC 700AU	131.9	mg/dl	Optimal : <100 near /above Optimal:100 - 129 Borderline High:130 - 159 High : 160 - 189 Very High :>=190
Serum VLDL Cholesterol Method: Calculated Machine: BECKMAN DXC 700AU	14.0	mg/dl	<30
Total CHOL / HDL Cholesterol Ratio Method: Calculated	3.61	Ratio	3.30 - 4.40
LDL / HDL Cholesterol Ratio Method: Calculated	2.36	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
HDL / LDL Cholesterol Ratio	0.42	Ratio	Desirable/Low Risk : 0.5 - 3.0 Border Line/Moderate Risk : 3.0 - 6.0 Elevated/High Risk: > 6.0
Non-HDL Cholesterol Method: Calculated	146.0	mg/dl	0.0 - 160.0


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* You have received this report as a part of your 'Dyslipidemia' evaluation.

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency.

Dyslipidemias means increase in the level of one or more of the following:

a. Total Cholesterol

b. The "bad" cholesterol or low density lipoprotein (LDL) and/or triglyceride concentrations

Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood

* Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation.

* Healthians labs report biological reference intervals (normal ranges) in accordance to the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) Guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal Lipid levels

*NCEP recommends the assessment of 3 different samples drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Hence a single result of Lipid Profile may not be adequate for clinical decision making. Healthians' counselling team will reach you shortly to explain implications of your report. You may reach out to customer support helpline as well.

*NCEP recommends lowering of LDL Cholesterol as the primary therapeutic target with lipid lowering agents, however, if triglycerides remain >200 mg/dL after LDL goal is reached, set secondary goal for non-HDL cholesterol (total minus HDL) 30 mg/dL higher than LDL goal.

*High Triglyceride and low HDL levels are independent risk factors for Coronary Heart disease and requires further clinical consultation.

*Healthians lab performs direct LDL measurement which is more appropriate and may vary from other lab reports which provide calculated LDL values.

Liver Function Test (LFT)

Serum Bilirubin, (Total)	0.67	mg/dl	0.3 - 1.2
Method: Diazo			
Machine: BECKMAN DXC 700AU			
Serum Bilirubin, (Direct)	0.16	mg/dl	0 - 0.2
Method: Diazo			
Machine: BECKMAN DXC 700AU			
Serum Bilirubin, (Indirect)	0.51	mg/dl	0.0 - 0.8
Method: Calculated			
Aspartate Aminotransferase (AST/SGOT)	31.30	U/L	3- 50
Method: IFCC			
Machine: BECKMAN DXC 700AU			
Alanine Aminotransferase (ALT/SGPT)	47.7	U/L	3 - 50
Method: IFCC			
Machine: BECKMAN DXC 700AU			
Alkaline Phosphatase (ALP)	84.80	U/L	43-115


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Test Name	Value	Unit	Bio. Ref Interval
Method: IFCC AMP Buffer Machine: BECKMAN DXC 700AU			
Gamma Glutamyl Transferase (GGT)	26.9	U/L	5 -55
Method: IFCC Machine: BECKMAN DXC 700AU			
Serum Total Protein	7.70	g/dl	6.6 - 8.3
Method: Biuret Machine: BECKMAN DXC 700AU			
Serum Albumin	4.80	g/dl	3.5 - 5.2
Method: Bromo Cresol Green (BCG) Machine: BECKMAN DXC 700AU			
Serum Globulin	2.90	gm/dl	2.0 - 3.5
Method: Calculated			
Albumin/Globulin Ratio	1.66	Ratio	1.2 - 2.0
Method: Calculated			
SGOT/SGPT Ratio	0.66	Ratio	0.7 - 1.4
Method: Calculated			

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.

AST levels increase in viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. ALT levels may also increase after a heart attack or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc.

Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum. Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

Please note that there is a change in the Biological Reference Interval for Alkaline Phosphatase (ALP) parameter. (Dated: 12.02.2026).


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DEPARTMENT OF BIOCHEMISTRY

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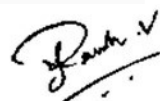
Test Name	Value	Unit	Bio. Ref Interval
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Kidney Function Test Advance

Serum Creatinine Method: Jaffes Kinetic Machine: BECKMAN DXC 700AU	0.74	mg/dl	0.6 - 1.6
GFR, ESTIMATED Method: Calculated (CKD-EPI)	120.43	mL/min/1.73m2	
Serum Uric Acid Method: Uricase Machine: BECKMAN DXC 700AU	5.1	mg/dl	3.5-7.2
Serum Calcium Method: Arsenazo Machine: BECKMAN DXC 700AU	10.3	mg/dl	8.8 - 10.6
Serum Phosphorus Method: Phosphomolybdate complex Machine: BECKMAN DXC 700AU	3.9	mg/dl	2.5 - 4.5
Serum Sodium Method: ISE (Indirect) Machine: BECKMAN DXC 700AU	144	mmol/L	136 - 146
Serum Chloride Method: ISE (Indirect) Machine: BECKMAN DXC 700AU	103	mmol/L	101 - 109
Blood Urea Method: Urease Machine: BECKMAN DXC 700AU	20.0	mg/dl	17 - 43
Blood Urea Nitrogen (BUN) Method: Calculated	9.4	mg/dl	7.92 - 20.03
Bun/Creatinine Ratio Method: Calculated	12.64	Ratio	
Urea/Creatinine Ratio Method: Calculated	27.03	Ratio	

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.



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Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR.

Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases.

Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.

EGFR-National kidney Disease Education program recommends the use of MDRD equation to predict/ estimate GFR in adults (≥ 20 years) with Chronic kidney disease. Calculated by serum creatinine may be less accurate due to certain factors like race, muscle mass, diet and certain drugs. In such cases, EGFR should be calculated using serum cystatin C.

Age (Years)	Male ML/min/1.73m ²	Female ML/min/1.73m ²
17-24	93-131	—
25-34	78-146	—
35-44	74-138	65-123
45-54	74-129	58-110
55-64	69-122	50-110
65-74	61-114	46-105
75-84	52-102	48-85



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NAFLD score

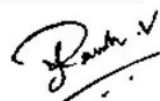
NAFLD score -5.08

Test Description :

- NAFLD Fibrosis Score (NFS) is a validated non-invasive tool used to assess the likelihood of advanced liver fibrosis in patients with suspected non-alcoholic fatty liver disease (NAFLD).
- It is derived from a combination of clinical and laboratory parameters.

NAFLD Score	Correlated Fibrosis Severity	Interpretation
< -1.455	F0-F2	low likelihood of advanced fibrosis
-1.455 – 0.675	Indeterminate score	further evaluation recommended
> 0.675	F3-F4	high likelihood of advanced fibrosis

- Non-alcoholic fatty liver disease (NAFLD) has become the most common cause of chronic liver disease worldwide and has a strong association with insulin resistance and metabolic syndrome.
- NAFLD encompasses a wide spectrum of liver pathology, ranging from simple steatosis without fibrosis to non-alcoholic steatohepatitis with varying stages of fibrosis and cirrhosis.
- In the absence of decompensated cirrhosis, liver biopsy remains the only reliable means to determine prognosis based on the severity of fibrosis. However, liver biopsy is an expensive and invasive procedure associated with a number of complications and prone to sampling error.
- Several non-invasive diagnostic scoring tests such as NAFLD fibrosis score (NFS), Enhanced liver fibrosis score (ELF) etc., have been developed to help determining the severity of liver fibrosis in lieu of liver biopsy.
- NAFLD fibrosis score (NFS) has a high negative predictive value and is useful as a first line screening tool to rule out advanced fibrosis, thereby reducing the need for liver biopsy in many patients. However, results should always be interpreted in conjunction with clinical findings and where indicated, additional investigations (e.g., imaging or elastography).



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Healthians Life Maximiser Plus Health Checkup Package - Male Couple

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CA 19.9 (Cancer Antigen 19.9)- Pancreatic Cancer

CA-19.9	8.70	U/mL	< 37
Method: CLIA			
Machine: UniCel DxI 800			

Clinical Significance: The CA19-9 test is used for the detection, diagnosis, and prognosis of pancreatic carcinomas, when supported by other investigations. It is useful for monitoring the response to therapy (e.g., postsurgical recurrence correlates with increased concentrations).

1. Increased levels can be seen in:

- Malignant (including but not limited to)- Carcinoma of pancreas, Hepatobiliary carcinoma, Gastric Carcinoma, Colon Carcinoma
- Non-cancerous (Benign) conditions (including but not limited to)- Cirrhosis, Cholangitis, Hepatitis, Pancreatitis, and Benign GI diseases, like Obstructive Jaundice, Thyroid Disease, Ovarian Disease, Diabetes, Biliopancreatic Diseases, Pneumonia, Renal Failure, and Autoimmune disease.
- It may also be elevated in elderly individuals, women, and individuals with low BMI.

2. Decreased levels can be seen in:

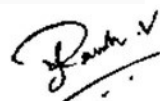
- After effective therapy or removal of the tumor

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies

CEA (Carcino Embryonic Antigen)

CARCINO EMBRYONIC ANTIGEN (CEA)	1.99	ng/mL	Non smokers < 3.00 Smokers < 5.00
Method: CLIA			
Machine: UniCel DxI 800			



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Test Name	Value	Unit	Bio. Ref Interval
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Note:

1. This test is not recommended for cancer screening in the general population.
2. False negative / positive results are observed in patients receiving mouse monoclonal antibodies for diagnosis or therapy.
3. Patients with confirmed carcinoma may show normal pre-treatment CEA levels. Hence this assay, regardless of level, should not be interpreted as absolute evidence for the presence or absence of malignant disease. The assay value should be used in conjunction with findings from clinical evaluation and other diagnostic procedures.
4. Persistently elevated CEA levels are usually indicative of progressive malignant disease and poor therapeutic response
5. The results obtained with different analytical techniques and different equipments cannot be used interchangeably due to difference in assay methods and reagent specificity.
6. In course of monitoring, the assay method preferably should not be changed.

Comment:

CEA is produced by cells during embryonic and fetal life and production ceases at birth. A very low serum concentration can be detected in healthy individuals. Increased CEA levels can be found in certain cases of cancer (Colorectal, breast, lung cancer etc.), but also in nonmalignant diseases. Serum CEA levels decrease after treatment & increase in the event of cancer recurrence, residual disease and metastases. The CEA test can be used as an additional test for prognosis and therapeutic monitoring of patients with diagnosed malignant carcinomas. Smokers have higher values than non smokers.

Clinical Use

Monitoring patients with Colorectal, Gastrointestinal, Lung & Breast carcinoma

Diagnosis of occult metastatic disease and / or residual disease

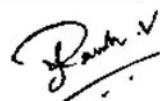
Increased values

- * Colorectal, gastrointestinal lung, pancreas, breast, and ovarian cancer
- * Some diseases like cirrhosis, pancreatitis, kidney failure, inflammatory bowel disease, peptic ulcer disease, chronic obstructive pulmonary disease (COPD), or an obstructed bile duct.
- * **Heavy smoking affects the test results.**
- * Please note test values may vary depending on the assay method used.

Clinical Significance: CEA is a glycoprotein involved in cell adhesion. It is normally produced in fetal gastrointestinal tissue, and its production stops before birth. Healthy adults normally have very low levels of CEA in their blood. Elevated levels may be associated with various malignant and non-malignant conditions.

1. Increased levels can be seen in:

- Malignant (including but not limited to)- Colorectal Carcinoma, Breast Cancer, Pancreatic Cancer, Lung Cancer, Medullary Thyroid Carcinoma, Ovarian/Cervical Carcinoma.
- Non-cancerous (Benign) conditions (including but not limited to)- Active Nonmalignant inflammatory diseases (especially of GI Tract) (Ulcerative Colitis, Regional Enteritis, Cholecystitis, Diverticulitis, Peptic Ulcers, Chronic Pancreatitis, Inflammatory Bowel Disease), Liver Disease (Alcoholic, Cirrhosis, Chronic



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Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:59PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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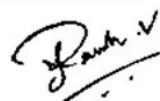
Test Name	Value	Unit	Bio. Ref Interval
Active Hepatitis, Obstructive Jaundice), Renal Disorder, Fibrocystic Disease of Breast, Smokers, Breast Cysts, Chronic Pulmonary Diseases, Mucinous Cystadenoma of the Ovary/Appendix, Lungs Issues, Ulcers, etc.			

2. Decreased levels can be seen in:

- Effective therapy or post removal of the tumor

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:55PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
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Ferritin

Ferritin	70.7	ng/ml	23.9-336.2
Method: CLIA			
Machine: UniCel DxI 800			

Ferritin estimation is useful in the diagnosis of iron deficiency anemia and iron overload.

Elevated ferritin levels also are observed in acute and chronic liver disease, chronic renal failure and in some types of neoplastic disease.

Increased levels seen in hemochromatosis, frequent blood transfusions with packed RBCs and alcoholic liver disease. Decreased levels seen in heavy menstrual bleeding, poor absorption of iron, iron deficiency anaemia and long term GI bleed.

Ferritin is an acute phase reactant and thus may be increased with inflammation, chronic infection, liver disease, auto-immune disorders and some type of cancers. Ferritin is not used to detect or monitor these conditions.

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Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 08:44PM
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF IMMUNOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Homocysteine

HOMOCYSTEINE	25.9	μmol/L	6 - 16
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Method: Immunoturbidimetric
Machine: BECKMAN DXC 700AU

Clinical Significance

Homocysteine is linked to increased risk of premature coronary artery disease, stroke and Thromboembolism and additionally alzheimer's disease, Osteoporosis, Venous Thrombosis, schizophrenia, Cognitive Deficiency and pregnancy complications.

High Values

Elevated Homocysteine may be due to increasing age, genetic traits, drugs, Renal dysfunction, and dietary deficiency of vitamins or smoking. To lower your Homocysteine, eat more green vegetables, stop smoking, Alcohol. Folic Acid helps lowering elevated levels.

Caution while interpretation

To get most accurate results, it is mandatory to separate serum immediately. In separated serum, Homocysteine remains stable for at least 48 hours at room temperature.



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 05:04PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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PSA Total (Prostatic Specific Antigen, Total)

PROSTATIC SPECIFIC ANTIGEN (PSA TOTAL)	0.77	ng/mL	0 - 4
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Method: CLIA

Machine: UniCel DxI 800

Clinical Significance: Prostate-specific antigen (PSA) is a protein produced by the cells of the prostate gland (both normal and malignant cells). The Total PSA test is primarily used in screening, diagnosis, and monitoring of prostate-related conditions.

1. Increased levels can be seen in:

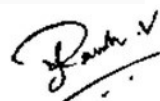
- Malignant (including but not limited to)- Prostate Carcinoma
- Non-cancerous (Benign) conditions (including but not limited to)- Prostatitis, Benign Prostatic Hyperplasia, Prostatic Ischemia, Acute Urinary Retention, Other conditions, like; Prostatic Massage, Cystoscopy, Needle Biopsy, TURP, Digital Rectal Examination, Radiation therapy, Indwelling Catheter, Vigorous Bicycle Exercise, Ejaculation causes transient increase, Acute Renal Failure, Acute Myocardial Infarction

2. Decreased levels can be seen in:

- Post 48 hrs. of ejaculation, Castration, Antiandrogen Drugs, Radiation Therapy, Prostatectomy, Drugs Therapy like Finasteride etc

Note:

- The diagnostic efficiency of any tumor marker depends on a variety of factors such as its sensitivity, specificity, positive predictive value (PPV) and negative predictive value (NPV). A single tumor marker test result should not be used in isolation for the diagnosis/ confirmation of any suspected malignancy. It should be used in association with clinical correlation/ imaging studies/ further investigations, under the supervision of a clinician only
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- Serial testing should be done to identify elevated levels caused due to transient physiological/ pathological factors
- Samples from patients receiving mouse monoclonal antibodies for diagnosis or therapy, may show falsely elevated or depressed values under specific conditions. Heterophilic antibodies in human serum, certain medications & non-specific protein binding may interfere with the test results
- Tumor marker test results are influenced by assay methods, reagent sensitivity and specificity, and should not be used interchangeably. If in the course of monitoring a patient, the assay method used is changed, additional testing should be carried out to confirm baseline values. All the results need to be followed up for correlation on similar platforms and methodologies



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Referred By	: Self	Report Generated On	: 22/Aug/2026 05:04PM	
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Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Profile (Total T3,T4, TSH)

Tri-Iodothyronine (T3, Total)	1.52	ng/ml	0.87 - 1.78
Method: CLIA			
Machine: UniCel DxI 800			
Thyroxine (T4, Total)	12.10	ug/dl	5.48-14.28
Method: CLIA			
Machine: UniCel DxI 800			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	6.820	μIU/mL	0.38 - 5.33
Method: CLIA			
Machine: UniCel DxI 800			

Pregnancy interval

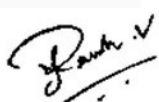
Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)

First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

Testosterone - Total


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Referred By	: Self	Report Generated On	: 22/Aug/2026 05:04PM
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF IMMUNOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
Testosterone, Total	4.63	ng/mL	1.98-6.79
Method: CLIA			
Machine: UniCel DxI 800			

Test Interpretation

*Total Testosterone reference intervals as per Tanner Stage:

Tanner Stage	Reference Interval – Male (ng/mL)	Reference Interval – Female (ng/mL)
Stage 1	0.02 – 0.23	0.02 – 0.10
Stage 2	0.05 – 0.70	0.05 – 0.30
Stage 3	0.15 – 2.8	0.10 – 0.30
Stage 4	1.05 – 5.45	0.15 – 0.40
Stage 5	0.65 – 8.0	0.10 – 0.40

Tanner staging should be assessed by the treating physician/ pediatrician.

Testosterone in males is secreted by adult leydig cells and is controlled by luteinizing hormone (LH). In females, it is produced in the ovaries, adrenal gland, and peripheral fatty tissues and has a serum concentration that is approximately 10-fold less than in males. In healthy adults, ~44% of circulating testosterone is bound to sex hormone binding globulin (SHBG), 50- 55% is bound to albumin & cortisol-binding globulin, & 2-3% is free; later three being bioavailable forms.

Increased in:

Adrenal and ovarian tumors, CAH, Polycystic ovary syndrome, ovarian stromal hyperthecosis, Idiopathic hirsutism

Decreased in:

Primary hypogonadism (e.g., orchiectomy), Secondary hypogonadism (e.g., hypopituitarism), Testicular feminization, Klinefelter syndrome, Cirrhosis, Chronic renal disease, Estrogen therapy

Limitations- Testosterone exhibits significant circadian variations, thus early-morning samples are recommended. Because SHBG concentrations can be influenced by many factors, tests assessing bioavailable and free testosterone is helpful in clinical decision-making.

Ref: Tietz textbook of clinical chemistry and molecular diagnostics, 5th edition

Wallach's interpretation of diagnostic tests, 11th edition

Note: Kindly note change in reference ranges with effect from 18/08/2023.

Vitamin B12

VITAMIN B12	141	pg/ml	222 - 1439
Method: CLIA			
Machine: UniCel DxI 800			

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency.

Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12 . The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Please note change in Biological Reference interval.

Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	29.41	ng/ml	30 - 100
Method: CLIA			

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Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 03:01PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 05:04PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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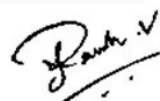
Machine: UniCel DxI 800

Biological Reference Ranges:

Deficiency	Below 20 ng/ml
Insufficiency	20 - 30 ng/ml
Sufficiency	30 - 100 ng/ml
Toxicity	Above 100 ng/ml.

The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs).

The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20-30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.



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Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:27PM
Referred By	: Self	Report Generated On	: 22/Aug/2026 10:14PM
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓
Sample Type	: Whole Blood EDTA	Report Status	: Final Report

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count Advanced

Haemoglobin (HB) Method: Spectrophotometry Machine: Yumizen H2500	16.9	g/dL	13.0-17.0
Total Leucocyte Count (TLC) Method: Impedance Machine: Yumizen H2500	6.9	10 ³ /uL	4.0-10.0
Hematocrit (PCV) Method: Calculated Machine: Yumizen H2500	50.7	%	40.0-50.0
Red Blood Cell Count (RBC) Method: Impedance Machine: Yumizen H2500	5.63	10 ⁶ /μl	4.50-5.50
Mean Corp Volume (MCV) Method: Derived from RBC Histogram Machine: Yumizen H2500	90.1	fL	83.0-101.0
Mean Corp Hb (MCH) Method: Calculated Machine: Yumizen H2500	30.0	pg	27.0-32.0
Mean Corp Hb Conc (MCHC) Method: Calculated Machine: Yumizen H2500	33.3	g/dL	31.5-34.5
RDW - CV Method: Derived from RBC Histogram Machine: Yumizen H2500	15.0	%	11.6-14.0
RDW - SD Method: Derived from RBC Histogram Machine: Yumizen H2500	36.00	fL	39.0-46.0
Mentzer Index Method: Calculated	16.00	Ratio	
RDWI Method: Calculated	240.05	Ratio	
Green and king index Method: Calculated	72	Ratio	

Differential Leucocyte Count



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Referred By : Self
Customer Since : 27/Jun/2023
Sample Type : Whole Blood EDTA

Barcode : D0342237
Sample Collected On : 22/Aug/2026 10:29AM
Sample Received On : 22/Aug/2026 02:27PM
Report Generated On : 22/Aug/2026 10:14PM
Sample Temperature : Maintained ✓
Report Status : Final Report

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
Neutrophils Method: Flow-Cytometry DHSS Machine: Yumizen H2500	51.4	%	40 - 80
Lymphocytes Method: Flow-Cytometry DHSS Machine: Yumizen H2500	34.2	%	20-40
Monocytes Method: Flow-Cytometry DHSS Machine: Yumizen H2500	8.5	%	02 - 10
Eosinophils Method: Flow-Cytometry DHSS Machine: Yumizen H2500	5.4	%	01 - 06
Basophils Method: Impedance Machine: Yumizen H2500	0.5	%	00 - 02
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC) Method: Calculated Machine: BECKMAN DXH 800	3.56	10 ³ /uL	2.0-7.0
Absolute Lymphocyte Count (ALC) Method: Calculated Machine: BECKMAN DXH 800	2.37	10 ³ /uL	1.0-3.0
Absolute Monocyte Count Method: Calculated Machine: BECKMAN DXH 800	0.59	10 ³ /uL	0.2-1.0
Absolute Eosinophil Count (AEC) Method: Calculated Machine: BECKMAN DXH 800	0.37	10 ³ /uL	0.02-0.5
Absolute Basophil Count Method: Calculated Machine: BECKMAN DXH 800	0.03	10 ³ /uL	0.02 - 0.10
Platelet Count(PLT) Method: Impedance Machine: Yumizen H2500	343	10 ³ /μl	150-410
MPV Method: Derived from PLT Histogram Machine: Yumizen H2500	8.7	fL	7 - 9

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Referred By	: Self	Report Generated On	: 22/Aug/2026 10:14PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
NLR (Neutrophil Lymphocyte Ratio) Method: Calculated	1.50	Ratio	
PLR (Platelet Lymphocyte Ratio) Method: Calculated	145	Ratio	

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <65 Beta Thalassemia Trait. For RDW Value >220 more likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

The neutrophil to lymphocyte ratio (NLR) is considered to be a sensitive indicator of infection, inflammation and sepsis, and a robust prognostic marker of disease severity.

The platelet-to-lymphocyte ratio (PLR) is a marker revealing shifts in platelet & lymphocyte cell counts due to acute inflammatory and pro-thrombotic states.



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Referred By	: Self	Report Generated On	: 22/Aug/2026 03:37PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Whole Blood EDTA	Report Status	: Final Report	

DEPARTMENT OF HAEMATOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
ERYTHROCYTE SEDIMENTATION RATE (ESR)			
ESR	4	mm/1st hour	0-10
Method: Modified Westergren Method			
Machine: Vesmatic cube 80			

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Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:56PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:30PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Yellow		Pale Yellow
Method: Visual			
Volume	20.00	mL	
Method: Visual			
Appearance	Clear		Clear
Method: Visual			

CHEMICAL EXAMINATION

Specific Gravity	1.015		1.001 - 1.035
Method: Dipstick-Ion exchange			
Machine: Urine Analyzer(URI-PLUS600)			
pH	5.5		4.5 - 7.5
Method: Automated/ strip Dipstick-Double indicator			
Machine: Urine Analyzer(URI-PLUS600)			
Glucose	Negative		Negative
Method: Automated/ strip Dipstick-oxidase peroxidase			
Machine: Urine Analyzer(URI-PLUS600)			
Urine Protein	Negative		Negative
Method: Automated/ strip Dipstick-Bromophenol blue			
Machine: Urine Analyzer(URI-PLUS600)			
Ketones	Negative		Negative
Method: Automated/ strip Sodium nitroprusside			
Machine: Urine Analyzer(URI-PLUS600)			
Urobilinogen	Normal		Normal
Method: Automated/ strip Dipstick-Ehrlichs Test			
Machine: Urine Analyzer(URI-PLUS600)			
Bilirubin	Negative		Negative
Method: Automated / strip (Diazonium salt)/ Fouchets test			
Machine: Urine Analyzer(URI-PLUS600)			
Nitrite	Negative		Negative
Method: Automated/ strip Dipstick-Griess test			
Machine: Urine Analyzer(URI-PLUS600)			
Blood	Negative		Nil
Method: Automated/ strip Dipstick-Peroxidase			

Ramesh V

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:56PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 04:30PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
Machine: Urine Analyzer(URI-PLUS600)			
Leucocyte Esterase	Negative		Nil
Method: Dipstick- Esterase			
Machine: Urine Analyzer(URI-PLUS600)			
MICROSCOPIC EXAMINATION			
Pus Cells	3-5	/HPF	0 - 5
Method: Microscopy			
Epithelial cells	2-3	/HPF	0 - 5
Method: Microscopy			
RBCs	Nil	/HPF	Nil
Method: Microscopy			
Casts	Nil		Nil
Method: Microscopy			
Crystals	Nil		Nil
Method: Microscopy			
Bacteria	Absent		Absent
Method: Microscopy			
Yeast Cell	Nil		
Others (Non Specific)	Nil		
Method: Microscopy			

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra.

Ketonuria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.

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SIN No:D0342237

Healthians Diagnostic Labs (A Unit of Expedient Healthcare Marketing Pvt. Ltd.)

4th Floor, 301, The Summit, 3rd A Main, 4th B Cross East of NGEF BDA Layout (Kasturi Nagar East), Ramamurthy Nagar Main Road, Bengaluru, Karnataka-560016.

(NABL Accreditation Certificate No. MC-4391)

Patient Name	: Amit Goyal	Barcode	: D0342237	
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Sample Type	: Urine	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Dr. Ramesh Vishwanath

DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



Patient Name	: Amit Goyal	Barcode	: D0342237	
Age/Gender	: 36Y 0M 0D /Male	Sample Collected On	: 22/Aug/2026 10:29AM	
Order Id	: 19852473167	Sample Received On	: 22/Aug/2026 02:25PM	
Referred By	: Self	Report Generated On	: 22/Aug/2026 02:49PM	
Customer Since	: 27/Jun/2023	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF SEROLOGY

Healthians Life Maximiser Plus Health Checkup Package - Male Couple

Test Name	Value	Unit	Bio. Ref Interval
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Hepatitis B Virus Surface Antigen (HBsAg)

Hepatitis B Virus Surface Antigen	Non Reactive		Non- Reactive
Method: Rapid chromatographic immunoassay			

This is a screening test ,If requires confirmation or correlation can be done by HBsAg neutralisation test .HBV viral load can be done post clinician evalutaion. This is a visual, rapid immuno chromatographic,one step immunoassay based on antigen capture for the qualitative detection of Hepatitis B Surface Antigen (HBsAg) in Human Serum or Plasma. Samples containing mouse monoclonal antibodies or heterophile antibodies can give falsely reactive or non-reactive results. False Reactive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

*** End Of Report ***


DR RAMESH VISHWANATH
MBBS, MD PATHOLOGY
HEALTHIANS LABS



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1. Test results released pertain to the sample/specimen submitted by the customer.
2. Test results are dependent on the quality of the samples received by the Lab.
3. The tests are carried out in the lab with the presumption that the sample/specimen belongs to the patient named or identified in the bill/test request form/booking ID.
4. The laboratory reports are not diagnostic. The reported results are for information only, and are subject to clinical correlation and interpretation by the consulting physician.
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ADVISORY

Health Advisory

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

23.81 Body Mass Index

5'11" Height (ft/in)

78 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

0cm



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED
NUTRITION

SUGGESTED NUTRITION

Do's

- Include whole grains in your diet like whole wheat bread and other products, brown rice or hand pounded rice, oats
- Have fresh fruits, green leafy vegetables and unsalted nuts and seeds
- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Have high protein, moderate fat diet with low fat milk, yoghurt or buttermilk
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds
- Include fruits like apples, berries and melons in your diet

Dont's

- Limit sugar intake
- Avoid high cholesterol and calorie dense foods
- Avoid the use of oil and avoid sauces and dressings
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Avoid cruciferous foods like cauliflower, cabbage and spinach
- Avoid soy products like soymilk or tofu
- Avoid red meat and organ meats
- Avoid refined sugars, processed foods and bakery items
- Reduce caffeine intake
- Decrease intake of colas and sugary drinks

SUGGESTED
LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Maintain ideal weight
- Stay active and maintain ideal weight
- Have regular exposure to sunlight
- Sleep well at night and do relaxing activities

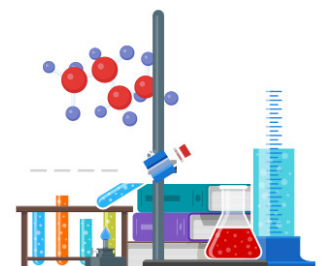
Dont's

- Avoid having long gaps in meals or skipping meals
- Avoid strenuous exercises
- Avoid smoking and alcohol
- Avoid overeating or calorie rich food
- Avoid overworking or being stressed for long time
- Avoid long periods of inactivity
- Avoid overexertion without having food or drink
- Avoid late night heavy meals

SUGGESTED
FUTURE
TESTS

SUGGESTED FUTURE TESTS

- Complete Hemogram - **Every 2 Month**
- Vitamin B12 Cyanocobalamin - **Every 2 Month**
- Iron Studies - **Every 2 Month**
- Folic Acid - **Every 2 Month**
- Thyroid Profile-Total (T3, T4 & TSH Ultra-sensitive) - **Every 1 Month**
- Testosterone Total - **Every 1 Month**
- Vitamin D Total-25 Hydroxy - **Every 2 Month**



HEALTH ADVISORY

Suggestions for Health & Well-being

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week. If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet. It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is **23.81**, which is on a normal side. Please follow recommended diet and maintain a healthy lifestyle and try to keep your BMI within the desired range. Keeping the right BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART



SUGGESTED BMI

Supplement Suggestions

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
LDL Cholesterol -Direct	HEARTUP
Cholesterol-Total, Serum	APPLE CIDER VINEGAR

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



HEARTUP

Improve your heart health, the natural way!

Lower your blood pressure and give your heart a healthy beat with HEART-UP, an all-natural supplement developed especially to promote good heart health. Harnessing the remedial properties of garlic, peepal, and cinnamon, this clinically proven natural supplement lowers your blood pressure, thus ensuring a healthy heart, which in turn means a healthy you.

If left unchecked, hypertension can lead to:

- Heart Failure
- Kidney Diseases
- Heart Attack
- Stroke
- Vascular Dementia

Infused with the ages-proven goodness of all-natural ingredients, HEART-UP is the perfect supplement to help you control hypertension or high blood pressure without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in HEART-UP present the following benefits:

Arjun Tree Extract

Reduces the risk of heart diseases with anti-hypertensive properties

Garlic

Helps manage blood pressure and lowers cholesterol

Peepal

Purifies the blood and boosts cardiac health

Jatamansi

Helps in alleviating anxiety, thus reducing the risk of high blood pressure

Cinnamon

Has anti-viral properties, reduces blood pressure, and lowers the risk of Type 2 diabetes



APPLE CIDER VINEGAR

Boosts Energy & Improves Immunity

Herbved Apple Cider Vinegar with Mother is 100% natural, raw, unfiltered and unpasteurized that has been fermented from Himalayan apples. It contains a substance called mother that consists of antioxidants, proteins, enzymes, and friendly bacteria that offers you a host of health benefits like losing weight, controlling High BP, Sugar & Cholesterol and improving digestion. It can work wonders on your hair making it soft and shiny, controlling dandruff & hairfall and can also prevent acne when applied as a toner externally on your skin.

Benefits of Apple Cider Vinegar:

- Helps in weight loss
- Helps control cholesterol levels
- Lowers blood pressure
- Controls hairfall and makes hair soft and shiny
- Improves digestion

Supplement Suggestions

Amit Goyal

Booking ID : 19852473167 | Sample Collection Date : 22/Aug/2026

Deficiency/Out of Range Parameter(s)	Suggested Supplement
TSH Ultra - Sensitive	THYRO FIX
Vitamin B12 Cyanocobalamin	VITAMIN B12

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



THYRO FIX

Here's nature's way to improve your thyroid function.

THYRO-FIX is a scientifically formulated and clinically proven all-natural supplement that helps strike the optimum balance of your thyroid levels. Whether hyperthyroidism or hypothyroidism, this ayurvedic supplement keeps your thyroid balanced and optimally functioning. Take the all-natural road to a healthy thyroid with THYRO-FIX.

Be it hyperthyroidism or hypothyroidism, untreated thyroid conditions can cause serious health issues, such as:

- Cardiovascular Diseases
- Brittle Bones
- Eye Issues
- Infertility
- Mental Health Concerns

Infused with the ages-proven goodness of all-natural ingredients, THYRO-FIX is the perfect supplement to promote and maintain good thyroid health, without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in THYRO-FIX present the following benefits:

Arjun Tree Extract

Decreases thyroid levels in hyperthyroidism to maintain hormonal balance

Ashwagandha

Increases thyroid levels in hypothyroidism to maintain hormonal balance

Anantmool

Anti-inflammatory & anti-oxidant properties reduce the symptoms of thyroid disorder

Asparagus

Regulates blood sugar levels & promotes heart health



VITAMIN B12

Make healthier blood the foundation of a healthier life!

Keep your blood clean and your nerve cells healthy with Vitamin B12. This essential supplement also protects against anemia, which causes persistent fatigue. Extracted from natural food sources, such as amla and beetroot, daily intake of Vitamin B12 can help in the formation of red blood cells.

A lack of vitamin B12 can cause dangerous health complications, such as:

- Memory Loss
- Vision Loss
- Ataxia or Loss of Physical Coordination
- Irreversible Neurological Problems
- Heart Diseases

Prevents Osteoporosis & Promotes Bone Health

Prevents Anemia & Promotes Red Blood Cell (RBC) Production

Alleviates Symptoms of Depression

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